



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : انشراح عزوز محمود الليثى

العنوان : ..

المحافظة : القاهرة

رقم البطاقة : 24710030101624 رقم التليفون : 01100305631

العمر (تاريخ الميلاد) : 03-10-1947 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: انشراح عزوز محمود اللش National ID: 24710030T01624	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



DM
مقلعة



جوستاف
روستاف
2026



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

مبادرة
رئيس الجمهورية
لدعم صحة المرأة المصرية

04/05/2026

MDT FORM

Name:	Arshirah Essuz Mohamoud Elisy	MRN:	6699
Age:	79 Y	ID:	24710030101624
Menopausal Sta	POST	Phone:	01004235196
NRS SCORE:	PS_2	Marital Status:	MARRIED
BMI:	35	Family History:	+VE
CO Morbidity:	HTN AND CARDIAC NOT ON FOLLOW UP	Reason Of MDT:	NEW CASE

Clinical Data:

BX:	10C G2
ER:	0/5
PR:	0/5
HER2:	0
K167:	20

R

L

Sonomammography:

RT BREAST LOQ MASS AT 6 CLOCK 3*2.8 cm, ACR + A
FEW LMS WITH INCREASED CORTICAL THICKNESS > 3MM LARGEST 1.3*1.3
CLINICALLY INCREASED SKIN THICKNESS AND RETRACTED NAC AND SKIN

Mets Workup:

CT CAP WITH C
BIL FEW PULMONARY NODULES FOR FOLLOW UP
PROMINANT PULMONARY TRUNK AND MILD CARDIOMEGALY (ECHO REQ)
DILATED CBD WITH NO OBSTR OR JAUNDICE
SMALL HIATUS HERNIA

History:

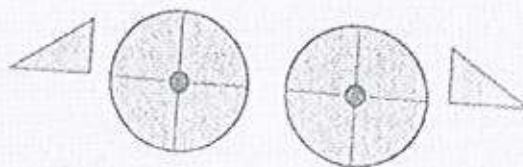
CLINICALLY T4B N1 FOCAL SKIN THICKNESS OVER THE MASS

Recommendations:

CARDIAC CONSULTATION
AND FOR POSSIBLE UPFRONT SURGERY AS PS NOT WELL FOR NACT

MDT Outcome & Plan

AS EF 67% AND THE CASE IS LOCALLY ADVANCED THERE IS A CLINICAL
BENEFIT OF NACT SO FOR NACT EC X3 TAXOL WEEKLING AFTER COUNCILING
OF THE PT AND SEE THE PERFORMANCE STATUS



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
	<i>[Signature]</i>	<i>[Signature]</i>			

Name: انشراح عزوز محمود الليثي			
MR. : 6699			
Detailed Medical Report			
			السيد
تجبة طبية وبعد,,			
		إيماء للطلب المقدم من	
		بقسم:	
بشأن تقرير طبي للمريض		رقم طبي:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
		تاريخ وساعة الدخول :	
		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient has a history of double primary malignancies. Thirteen years ago a hemicolectomy was performed for colon cancer. The patient subsequently developed right-sided breast cancer, staged cT4b N1 M0, with bilateral opulent nodules on imaging. A thyroid nodule was identified as TI-RADS 4; fine-needle aspiration yielded an inadequate sample (67 % cellularity). The breast tumor was estrogen-receptor positive (8/8), progesterone-receptor negative (0/8), HER2-negative, and Ki-67 20 %. The treatment plan included neoadjuvant chemotherapy with weekly taxol, epirubicin (unavailable), adriamycin 100 mg, and cyclophosphamide (endoxan) 1000 mg, with close monitoring of cardiac function (EF 67 % on exam) and assessment of tolerability. The patient returned for cycle 2 (C2) of adriamycin and endoxan, with supportive therapy; vital signs were weight 80 kg, height 155 cm and were stable. For cycle 3 (C3) of adriamycin and endoxan, laboratory values remained stable (weight 80 kg, height 155 cm). A bone scan revealed a solitary osseous nodule in the right third rib, prompting MRI for further characterization. Hemoglobin was 7.6 g/dL, prompting a blood transfusion and repeat CBC. Physical examination showed edema of the right breast, a 5 × 4 cm mass, a 2 × 2 cm right axillary lymph node, peau d'orange, and nipple retraction. A chest CT was ordered to assess the rib lesion. The patient received week 1 (w1) of taxol 120 mg (70 mg/m²) despite poor performance status; tolerance was to be reassessed the following week. Cardiology consultation was obtained after a decrease in ejection fraction to 37 % while on week 1 taxol 120 mg. On examination during week 2 (w2) of taxol 120 mg, right-breast edema and induration were noted. Week 3 (w3) taxol was pending pending a repeat chest CT and a new echocardiogram after three weeks. Due to a hemoglobin of 8 g/dL and a 10 % dose reduction, the patient received week 2 taxol 105 mg, one prophylactic dose of Neupogen, and a blood transfusion. Because the total lymphocyte count was 1.4 × 10⁹/L (with an absolute neutrophil count of 0.4 × 10⁹/L), taxol was postponed for one week; if neutropenia persisted, the regimen would be deemed intolerable. Multidisciplinary discussion considered neoadjuvant hormonal therapy. Chest CT was clear, with a normal thoracic cage and no pulmonary nodules. During week 4 (w4), examination revealed a right retro-areolar 3 × 3 cm mass with skin dimpling, nipple retraction, and a 1 × 2 cm right axillary lymph			
تحريرا في ,			
		التاريخ : 19-09-2026	
		الطبيب المعالج : dina makram	
يعتمد ,			

node. Taxol 105 mg was administered, and prophylactic Neupogen was given every two weeks.

During week 5 (w5), the same right retro-areolar mass and skin changes persisted; taxol dose was reduced to 100 mg and two prophylactic doses of Neupogen were administered.

During week 6 (w6), examination showed a right axillary lymph node 2×2 cm and the retro-areolar mass unchanged at 3×3 cm. Taxol 100 mg and one prophylactic Neupogen were given; week 7 (w7) taxol was planned. The patient has now completed seven weeks of weekly taxol.

ECHO REPORT

NAME: انشراح عزوز محمود

DATE: 16 / 3 / 202

ID NUMBER: ٣١٩ / ٢٥

SEX: ♀ AGE: ٢٨

F	67.1	55-75%	PWT	1.0	0.6-1.1cm
/EDD	5.0	3.5-5.8cm	AO.R	2.9	3-3.7cm
/ESD		2.4cm	LA	4.5	2-4cm
		25-45%	RV		0.7-2.3com(supine)
NT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

-V Dimensions

-V Wall Thickness

-V Diastolic Function

WMA

-A and Ao Root Dimensions

Mitral Valve

Aortic Valve

Tricuspid Valve

Pulmonary Valve

Right Side and Systolic Function

Intracardiac Masses and Effusion

Single Mitral (E) wave Absent A wave
for ECGs correlation

Dilated LA

Mild MR

Scl. AV with no sig systolic gradient with mild A.R

Mild TR EPRSP 50 mmHg

Mild

Dilated R.V with preserved systolic function

DR

Signature

Patient Name : الأستاذ / انشراح عزوز محمود	Request No. : 13005398
Patient No. : 24710030101624	Request Date : 10/03/2026
Path No. : C-48/26	Verification Date : 11/03/2026
Sex : Female	Financial Category : جوستاف روسي - مبانره
Age : 78 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

Clinical Data: Neck U/S showing left thyroid lobe moderately suspicious nodule (TIRADs 4).
Nature of specimen: Left thyroid lobe nodule, U/S guided FNA, Cytology.

Gross specimen:

Four alcohol fixed smears were received & then they were H&E stained.

Microscopic examination

Adequacy: Inadequate smears.

Cellularity: Hypocellular.

Colloid: Present.

Follicular cells: Not presented.

Other cellular elements: Only red blood cells & lymphocytes.

Diagnosis

Left thyroid lobe nodule, U/S guided FNA, Cytology:

Unsatisfactory smears with no follicular cells, Bethesda category I.

Comment: The BETHESDA System for reporting thyroid cytopathology: Diagnostic categories:

Non-diagnostic or Unsatisfactory.

. Benign. Consistent with a benign follicular nodule (includes adenomatoid nodule, colloid nodule, etc.).

Consistent with chronic lymphocytic (Hashimoto) thyroiditis in the proper clinical context.

Consistent with granulomatous (subacute) thyroiditis.

I. Atypia of undetermined significance or follicular lesion of determined significance.

✓. Follicular neoplasm or suspicious for follicular neoplasm.

✓. Suspicious for malignancy.

✓. Malignant.

Verified by :

Dr.Duaa Mohamed

Printing Date : 16/03/2026 09:37:22

Page 1 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

atient Name : الأستاذة / انشراح عزوز محمود
atient No. : 24710030101624
ith No. : C-48/26
ex : Female
ge : 78 Y
aferreing Doc :

Request No. : 13005398
Request Date : 10/03/2026
Verification Date : 11/03/2026
Financial Category : جوسفاف رومبي - مبادره
Department : Out Patient
Registered by : .marwa mohamed youss

Pathology Report

Signature
Dr. Walaa Ibrahim Ghoneim.

Verified by :
Dr. Duaa Mohamed

Printing Date : 16/03/2026 09:37:22

Page 2 of 2

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٢٦٩٩

الدكتور

جسین عبد الحزیز یس

استاذ الأشعة التشخيصية

كلية طب الأزهر

Name: Mrs. Ensherah Azouz Mahmoud.

Date: 18-Feb-26 8:41:56 PM Age: 78Y.

Dear Colleague Prof Dr.

Real time thyroid gland ultrasonographic study using superficial 10 MHz linear probe has revealed:

- * The right thyroid lobe is enlarged in size shows large well defined mixed cystic and solid nodule with solid component is isoechoic, it measures about 3x3.5cm, no retrosternal extension.
 - * The left thyroid lobe and isthmus average of uniform homogenous echopattern and of smooth outline. Left thyroid lobe isoechoic solid nodule with macro calcifications, it measures about 15.5x16.5mm
 - * The thyroid gland measurements are:
 - o Rt. lobe = 30 mm in AP diameter.
 - o Lt. lobe = 20 mm.
 - o Isthmus A.P. = 3 mm.
- The thyroid capsule is intact with no abnormal vascularity thyroid parenchyma using Doppler study.
- * No evidence of retrosternal extension noticed.
 - * The neck vessels are of normal course and calibers.
 - * No evidence of suspicious lymphadenopathy noted.

Impression:

- ☑ Right thyroid non suspicious mixed cystic and solid nodule (TR2).
- ☑ Left thyroid lobe moderately suspicious nodule (TR4) for follow up.

Much Obligated, Yours,
Dr. Ayman Diab (MSc).

Name : Aosharah Azzouz Mahmoud El-Laythi Date : 04-01-2026 10:43
 Branch : Maadi Medical No. : 288141
 Ref.Physician Prof: Goustav Roussey Egypt Ins.Company : Walkin

DIGITALIZED LOW DOSE SOFT TISSUE MAMMOGRAPHY WITH COMPLEMENTARY BREAST US EXAMINATION OF BOTH BREASTS REVEALED:

- ACR A: predominant fatty breast parenchyma.
- An ill-defined speculated hyperdense lesion is seen at the LOQ of the right breast corresponding by US to an ill-defined globular shaped heterogeneous hypoechoic lesion is seen at 6 o'clock surrounded by echogenic halo and causing posterior shadowing. It measures 3x2.8 in its max dimensions.
- Few enlarged right axillary LNs one of them showing increased cortical thickness (>3mm) and effaced fatty hilum. It measures 1.3x1.3 cm.
- No pathological microcalcification clusters.
- No left breast solid or cystic lesions.
- Normal central ducts system.
- Normal skin thickness and contour as well as nipple areola complexes.
- left non specific axillary lymph nodes showing preserved fatty hila.

Opinion:

- Right breast UOQ nialignant looking lesion associated with ipsilateral axillary lymphadenopathy (BIRADS V); for pathological correlation.

Much Obliged;
 Dr. Sarah Gamal, MSc.

Dr. Sara Gamal